

DS2444B — THE MEDICAL REPORT: A COMPLETION GUIDE

A clinical, question-by-question guide for the health professional completing the Disability Income Assistance Medical Report, with templates keyed to the eligibility test. Rebuilt July 21, 2026 against the AISH Manual, DIA Program Policy, last updated July 2, 2026.

WHAT CHANGED IN THIS EDITION

Nurse practitioners may complete this report. The policy manual states the report is completed by the applicant's physician *or nurse practitioner*, without restriction as to section. Earlier editions of this guide reflected a physician-only requirement. That was accurate when written and is no longer.

The statutory test is in two stages, not one. Severe disability is defined once and applies to both programs. The employment limb is what separates AISH from ADAP. Earlier editions collapsed these. Page 2 sets out both.

Permanence is not the only route. The definition reads *permanent or likely to continue for at least two years*. A two-year horizon satisfies the durational element.

Psychological reports count. The definition expressly contemplates substantiation through relevant medical *or psychological* reports.

Cost. Where the program requests additional medical information, the program pays. And for a client transitioned to ADAP in July 2026 seeking reassessment for AISH, the government covers one Medical Report, with no time limit.

BEFORE YOU USE ANY TEMPLATE ON THESE PAGES

Every template here is scaffolding, to be completed to the true clinical picture and no further. Delete any clause that does not fit this patient.

Accuracy is the strongest position and the one that withstands review. An overstated report can be challenged and can cost a patient the case they truly have.

Your signature certifies your clinical opinion. These templates exist to put your real findings into the language the determination weighs, not to supply findings you did not make.

THE TEST THE REPORT MUST SPEAK TO

Stage one, severe disability. Identical for both programs. A substantial impairment of mental or physical functioning or both, that is **permanent or likely to continue for at least two years**, the existence and likely duration of which has been diagnosed and substantiated through relevant medical or psychological reports, **and for which all relevant treatment that could lessen the impairment has been completed and where no improvement is expected.**

Stage two, the employment limb. Assessed without considering any factor other than the severe disability and its impact on ability to participate in employment.

AISH — whether the severe disability **permanently prevents** employment.

ADAP — whether it **substantially impedes** employment **continuously or episodically.**

THE TWO FRAMINGS THAT DECIDE WHICH PROGRAM

The accommodation trap. Accommodation language reads as capable with support, which is the ADAP finding. Where clinically true, state that no accommodation renders competitive employment reliable or sustainable. An employer can adapt a workstation; none can adapt to unpredictable incapacity.

The word episodically. It appears in the ADAP definition and not in the AISH one. Recording a condition as episodic and nothing further maps the patient onto ADAP by the statutory wording. Where the clinical picture includes continuous baseline impairment as well as discrete exacerbations, record both. The daily floor is often the stronger finding and the one most often left out.

The second limb is where files are lost. All relevant treatment completed, no improvement expected. Address it expressly. Where no disease-modifying option exists, say so — a blank reads as treatment still pending. Where therapy is lifelong maintenance rather than remedial, say that: ongoing maintenance for an incurable condition is evidence of permanence, not against it.

RELATIONSHIP WITH APPLICANT (Q1–Q4)

Q1: confirm Alberta registration; give discipline and registration number. Q2 to Q4: treating duration, last contact, frequency. Longer regular treating relationships carry evidentiary weight. A specialist seeing the patient episodically should say so; it frames the scope of the opinion.

DIAGNOSIS AND MEDICAL HISTORY (Q5–Q10)

Q5. All conditions, primary and secondary, with codes and onset dates. Early or congenital onset supports the durational element. Include comorbidities that compound function even if individually sub-severe; cumulative burden is what is adjudicated.

Q6. Etiology, classification, stage or grade, and the specific symptoms per diagnosis.

[Condition] – [etiology/classification]. Symptoms this patient experiences: [list]. Functional consequence: [what the symptoms prevent]. Onset [mm-yyyy]; course [static / progressive / relapsing].

Q7. The central narrative field. Tie each condition to a duration and a functional restriction, and close on durability and impact on earning capacity.

[Patient] has a [X]-year documented history of [condition(s)], first presenting [mm-yyyy], with [stable / progressive] course despite [treatments tried]. The impairment restricts [functions]. These restrictions are [continuous / unpredictable and recurrent] and are not resolved by workplace accommodation, because [the limiting factor is unpredictable incapacity, not the work environment]. The impairment is [permanent / likely to continue well beyond two years]. In my opinion it permanently prevents this patient from participating in employment.

Q8–Q10. Specialist consultations, diagnostics, imaging, labs, hospital admissions with dates and reasons. Attaching documents pre-empts a later request for further information — and where the program requests it, the program bears the cost.

DEGREE OF IMPAIRMENT — ACTIVITIES OF DAILY LIVING (Q11–Q40)

Rate each item to the true clinical picture: 0 none, 1 mild, 2 moderate, 3 severe or unable. Do not under-rate to appear conservative. An accurate 2 or 3, with a line in the describe column, carries the file.

NOTE VARIABILITY — DO NOT AVERAGE IT AWAY

Complete the describe box wherever a rating is 2 or 3. For fluctuating conditions write the range, not the mean: 3 on flare days, 1 between — not a blended 2. A good day is not the measure; the pattern is. Averaging erases exactly the evidence that matters.

Q40 – Across mobility, cognition, personal care and independent living, this patient's cumulative impairment is [moderate-severe]. Key drivers: [items rated 2-3]. On [flare / bad] days, [patient] requires [assistance/supervision] for [tasks]. The combined effect is that [patient] cannot independently maintain the routine and reliability that sustained employment requires.

LEVEL OF IMPAIRMENT — MENTAL HEALTH (Q41–Q68)

Same rating discipline across mood, thought and perception, cognition, behaviour, orientation, speech, somatic and safety. Fields that most often carry these files: Q50 to Q52 attention, concentration, comprehension; Q53 executive function, pivotal for acquired brain injury and neurodevelopmental profiles; Q57 memory, specifying long, short or working; Q60 amotivation, Q59 impulse control, Q43 emotional dysregulation. Q67 is a clinical safety field; complete it from your assessment as you would any risk documentation.

Q68 – Cumulative mental-health impairment is [moderate-severe], driven by [items rated 2-3]. [Executive dysfunction / cognitive deficits / mood instability] mean [patient] cannot reliably [initiate and sequence tasks / sustain attention / regulate under ordinary workplace demand] without continuous external structure. These deficits are [longstanding / organic] and persist [despite treatment]. They are not correctable by accommodation, as they affect the capacity to self-organize and self-sustain that any employment presupposes.

REMEDIAL THERAPY (Q69–Q74) — the second limb

This is the field the statutory definition turns on: all relevant treatment that could lessen the impairment has been completed and no improvement is expected. Answer so the record closes that gate where it is clinically closed. Where a treatment is nominally available but inaccessible within a clinically relevant timeframe, say so — it is not meaningfully available.

Q71 – Current management: [plan], initiated [mm-yyyy], reviewed [interval]. Available interventions have been [tried / exhausted] with [partial / no] functional gain. [Remaining options are limited to symptom management and maintenance, not restoration of work capacity. / No disease-modifying treatment exists for this condition. / Further options are inaccessible within a clinically relevant timeframe: [waitlist / barrier].]

Q72–Q73. Compliance. An unexplained non-compliant can sink an otherwise strong file. Legitimate reasons belong on the record.

Q72-73 – Adherence is limited by [cost / side-effect intolerance / access / the disability itself, e.g. executive dysfunction impairing medication management]. This is a consequence of the condition and circumstances, not refusal of care. [Where trialled and not tolerated: these options are therefore closed to this patient.]

Q74 – No further treatment is anticipated that would be expected to restore this patient's capacity for reliable, sustained employment. Management is directed at [symptom control / stability / prevention of deterioration], not functional recovery.

PROGNOSIS (Q75) — the determinative field

Four options: Temporary, Episodic, Permanent, Undetermined.

READ THIS BEFORE TICKING EPISODIC

Permanent is the finding the AISH determination rests on, and it is available wherever the impairment is permanent *or likely to continue for at least two years*. The durational element does not require certainty of lifelong course.

Episodic is the word used in the ADAP definition and not in the AISH one. Where it is the accurate answer it should be given — but quantify frequency, duration, severity and residual limitation between episodes, and record any continuous baseline impairment alongside it. Unpredictable recurrence is itself a barrier to employment and should be stated as one.

Undetermined should be avoided where the record supports a durational finding. An ambiguous prognosis defaults the file toward the lower program.

Q75 – Permanent. [Condition(s)] are longstanding and not expected to improve with further treatment. Prognosis for return to reliable, sustained employment is poor. [For episodic presentations: episodes occur [frequency], last [duration], severity [level], onset unpredictable; between episodes the patient retains [residual limitation].]

ADDITIONAL COMMENTS (Q76) — the closing synthesis

Q76 – In summary, [patient] lives with [severe condition(s)] producing cumulative impairment across [physical / cognitive / mental-health] domains. The impairment is [permanent / likely to continue well beyond two years]. All relevant treatment that could lessen it has been [completed / exhausted / is unavailable], and no improvement is expected. The limitations are [continuous / unpredictable] and are not correctable by workplace accommodation. In my clinical opinion, this severe disability permanently prevents [patient] from participating in employment.

WHAT HAPPENS TO THIS REPORT

An adjudicator reviews it. Where it sufficiently demonstrates that the applicant may have a severe disability resulting in a permanent inability to work, the adjudicator refers the file to the **Medical Review Panel**, a panel of health professionals who conduct an independent review and determine AISH medical eligibility. That determination is final and cannot be appealed.

A decision not to refer the file to the panel can be appealed, to the Medical Appeal Panel, within thirty days. So a report that stops short of the AISH threshold does not close the matter — but the appeal panel may consider only information the program already held. Anything further must reach the program before the appeal is filed. Completeness now is worth more than any submission later.

Medical eligibility is assessed **last**, after general and financial. Where an applicant is denied on an earlier criterion, this report may never be read.

RECORDS AND CERTIFICATION

Netcare is not the full record. It holds labs, imaging, dispensed medications and hospital records, not community clinic office notes. Where relevant history sits outside Netcare, draw on the clinic chart, pharmacy record and specialist correspondence so the report is complete rather than partial.

Certification. Completing the report certifies it contains your clinical findings and medical opinion. Provide name, registration number, contact details, date and signature.

Sourced throughout from the Government of Alberta AISH Manual, DIA Program Policy, sections last updated July 2, 2026, and the ALSS Adjudication Guide, July 2026. General information, not legal or medical advice. Free to share, print and post.